



TO SEND OUT

Conversation: Dr Yusuf Shittu, MamaDoc

Revised once we knew who he actually is. Met at the APPG, 22 September 2026.

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The Founder, Vytalix

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He is an NHS GP in Leicester and the **co-founder and chief executive of MamaDoc**, which he built with Dr Nimat after they became parents in the UK away from family. MamaDoc runs antenatal classes, education and WhatsApp support for Black, Brown and migrant parents. That makes him three things at once, and you have to hold all three in your head.

He is	So
A practising GP and three-time founder	His criticism is worth more than anyone's in that room
Building in maternity, for the same women	Do not hand him your roadmap on a first meeting
Running his own company	He will never be your clinical safety officer. Drop that ask entirely

The honest read on whether he is a competitor

Today, no. MamaDoc sells to and serves **women directly**: community, classes, information, a human on WhatsApp. MaternaLink sells to **hospitals and programmes**: coordination, monitoring, escalation, a record.

He owns the demand side. You are building the supply side. Those fit together well.

But he is one product decision away from overlapping with you, and you are one decision away from overlapping with him. Both of you know it. Say the useful parts out loud and keep the build details to yourself. That is not dishonesty; it is what every founder does on a first meeting, and he will be doing exactly the same.

What to say, now that you know

Lead with his company. Not yours. It is the sincerest thing you can do and it is also correct.

"You're MamaDoc, aren't you? I've just been reading it. Can I ask you the thing I most want to know — what do the women actually message you about? Not what you expected. What they actually send."

That question is gold and only he can answer it. He has the real message log from the exact population every report in this building is about. Write down what he says.

Then the one that opens the partnership

"Here's my problem. I'm building the hospital side — coordination, escalation, a record of what was said. But I have no trust from women and you have all of it. Does the hospital side even matter to the women you talk to, or is that me solving the wrong end of it?"

You have just told him the shape of a partnership without proposing one. If he is interested, he will take it from there. If he is not, you have learned something important for free.

If he asks what MaternaLink does

Three sentences. Category, not features.

"Care coordination for maternity, sold to hospitals. Appointments, symptoms, and monitoring for the women who need watching. There was a prediction engine in the old version and I pulled it out, because it would make it a medical device and I can't back the claims."

Then stop and turn it back: "How did you handle the line between support and clinical advice?"

You're a GP running a parent service — you must have hit that early."* That is a real problem

he has definitely solved and he will enjoy being asked.

The two questions still worth asking

"How did you find your clinical safety officer — or did you structure MamaDoc so you didn't need one?"

"Is there a route into the Clinical Entrepreneur Programme for a founder who isn't a clinician, or does it need a clinical co-founder?"

Both are genuine. Neither gives anything away.

What not to say now

Do not	Why
"Will you be my clinical safety officer?"	He is a chief executive of a maternity company. It is the wrong ask and it shows you had not looked him up
Your module list, prices, or the government plan	He is building in maternity. Ordinary commercial care, nothing personal
"We could merge / you should join me"	Ten minutes in. It reads as desperate and it insults what he has already built
"I didn't know you had a company"	You do now. Never let him find out you researched him and pretended otherwise
Anything about the raise	He is a clinician and a founder. He will hear it as the wrong priority

If he asks something you would rather not answer, say so plainly: "I'd rather not go into

that bit yet, if you don't mind."* Every founder respects that answer and nobody respects

a vague one.

Close

"This is the most useful ten minutes I've had on this. Can I connect, and send you one paragraph on what I change after tonight? No deck, no pitch."

The message tonight

Yusuf — Kelvin, from the Black Maternal Health group this afternoon. I've spent the evening on MamaDoc. What you said about [the specific thing] is the part I had wrong. I build the hospital side of this — coordination and escalation, sold to trusts. You have the women's trust and I don't. That may be worth a conversation at some point, or it may not, and either is fine. Either way, thank you for being straight with me.

Short. No attachment. It names the fit without asking for anything, which is the only version a busy chief executive replies to.

Before any second conversation

1. Read mamadoc.co.uk properly. All of it.
2. Check MamaDoc at Companies House. Know whether it is a company, a CIC or a charity before you propose anything, because it changes what a partnership can even look like.
1. Work out what you would actually offer them. Do not arrive with a vague wish to work together.
2. Decide, in advance, what you will not discuss.